Medicare Advantage Dementia Contract Summary

SYNTHETIC DOCUMENT. Fabricated for a demo. Northlake Health is fictional; aggregate planning figures only, no patient-level data.

Prepared by: Revenue Cycle and Managed Care Contracting. **Scope:** Medicare Advantage terms that affect anti-amyloid therapy and memory care. Payer names are fictional placeholders. The Medicare summary reflects public CMS policy in general terms; check the current national coverage determination.

Why this matters

Medicare Advantage accounts for 33% of memory clinic visits; traditional Medicare fee-for-service accounts for 41%. Medicare Advantage plans must follow the national coverage policy but can add utilization management.

Coverage baseline: CMS registry requirement

CMS covers FDA-approved anti-amyloid monoclonal antibodies under coverage with evidence development (CED). For traditional FDA approvals, the treating clinician must take part in a qualifying registry. Both contracted Medicare Advantage plans apply this requirement; missing registry data puts reimbursement at risk.

Prior authorization rules

Plan	Prior authorization	Turnaround / renewal
Payer A (Medicare Advantage)	Amyloid confirmation, recent baseline MRI, APOE e4 counseling	7 to 14 days
Payer B (Medicare Advantage)	Prior authorization with monitoring MRI reports attached	Re-authorization every 6 months

Reimbursement notes

- Payer A pays infusion administration at 102% of the Medicare physician fee schedule.
- Payer B pays infusion administration at 98% of the Medicare physician fee schedule.
- Drug reimbursement for both plans follows the Medicare Part B rate; contracting is modeling 340B eligibility by site before launch.
- Payer B denies re-authorization when monitoring MRI reports are missing; there is no retroactive approval for infusions given after the authorization lapses.

Claims and timing

- Timely filing: Payer A allows 180 days from the date of service; Payer B allows 90 days.
- Payer A contract term renews on 2027-12-31; Payer B renews on 2027-06-30.

Operational requirements

- Build prior-authorization templates for each payer and a registry-submission workflow.
- Add 1 authorization specialist for neurology infusion.
- Track denials by reason code; missing MRI documentation was the top denial reason for peer systems.